

Session Objectives

- Identify and locate key cranial landmarks.
- Describe the normal physiologic motion at the Sphenobasilar Junction (SBJ), including:
- Axes of motion
- Movement of the sphenoid and occiput during cranial flexion and extension
- Participate in a palpation session focused on whole-body sensing, starting from the feet.
- Demonstrate appropriate cranial hand contacts with proper hand/finger placement and physician ergonomics for each cranial contact (Vault Contact, Fronto-Occipital Contact, Becker and Modified Becker (Base) Contacts) on multiple partners to develop diagnostic sensitivity.
- Perform and understand the OA Decompression technique for treating the occipitoatlantal (OA) joint.

Identify the following:

Bones

A. Frontal

1. Metopic suture
2. Glabella
3. Frontal eminences
4. Supraorbital notch/foramen
5. Supraorbital margin
6. Nasion

B. Zygoma

C. Sphenoid

1. Greater wing
2. Lesser wing
3. Basi-sphenoid
4. Sella turcica
5. Clinoid processes

D. Temporal

1. Zygomatic process
2. Mastoid process
3. Squamous portion
4. External auditory meatus

E. Parietal

1. Superior and inferior temporal lines
2. Squamous portion
3. Parietal prominences

F. Occiput

1. Nuchal lines
2. Inion
3. Squamous portion
4. Basi-occiput
5. Condyles
6. Confluence of sinuses
7. Transverse sinuses

G. Ethmoid

1. Crista galli

H. Maxillae/ Mandible

Foramina:

- A. Foramen Magnum
- B. Jugular
- C. Stylomastoid
- D. Supra & Infraorbital

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Sutures

A. Lambdoid

B. Sagittal

C. Coronal

D. Parietomastoid

E. Occipitomastoid

F. Parietotemporal

G. Fronto-sphenoidal

H. Temporo-sphenoidal

Landmarks

A. Bregma

B. Lambda

C. Pterion

D. Asterion

E. Spheno-squamous pivot

The Five Phenomena of the PRM

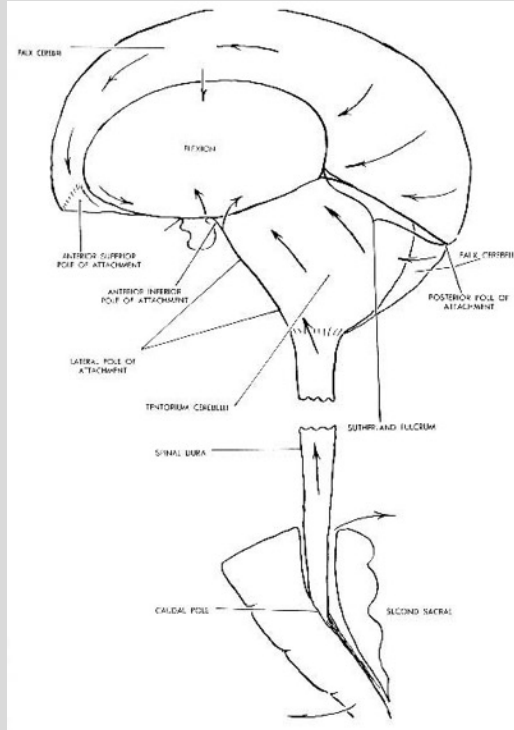
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- The inherent Motuility of the brain and spinal cord
- The Fluctuation of the cerebrospinal fluid and potency of the Tide
- The Mobility of the dural membranes
- The Mobility of the cranial bones
- The Involuntary Mobility of the sacrum between the ilia

William Garner Sutherland, Student of Andrew Taylor Still



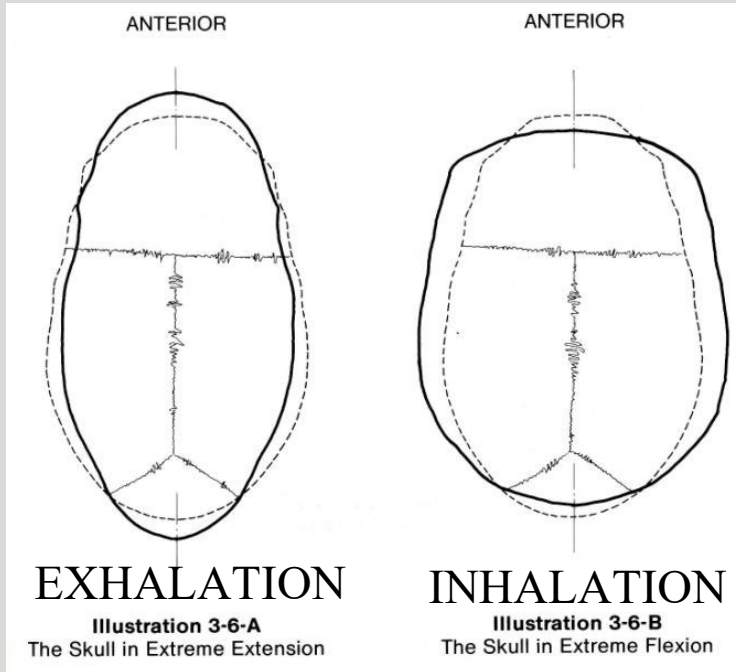


The mobility of the Dural membranes.

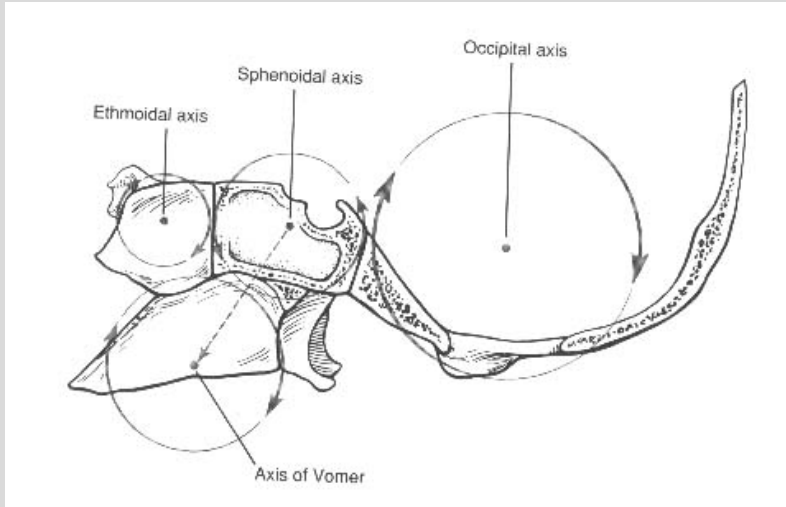
Act as check ligaments, guiding and limiting the movement of the cranial bones.

Motion is arcing, like that of a sickle.

Operates from a fulcrum, found along the straight sinus



The movement of the primary respiratory mechanism is an alternating change in shape of the cranium. During inhalation phase, the cranium widens transversely, and narrows both A/P and vertical dimensions. The movement of the PRM may be palpated throughout the body



Mobility of the cranial bones and sacrum:

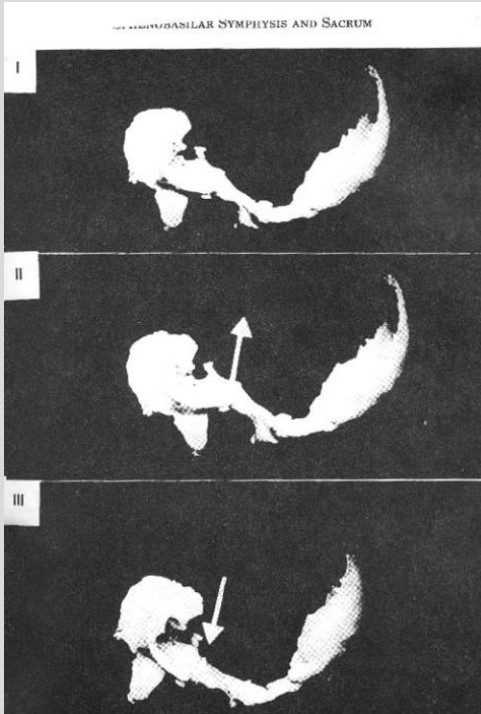
- Midline Bones
 - Flexion and Extension
- Paired bones
 - External and Internal rotation



Flexion Phase



Extension Phase



The rotation of the midline bones during inhalation and exhalation phases of the PRM is called flexion (inhalation phase), and extension (exhalation phase).

During flexion, the sphenobasilar junction moves superiorly (fig. II), and during extension, it moves inferiorly (fig. III).

This produces the vertical shape change associated with the phases of the primary respiratory mechanism.



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Inhalation phase:

Flexion midline bones,

External rotation paired bones

Sense of broadened transverse
diameter, but narrowed A/P and
vertical diameters



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Exhalation phase: Extension
midline bones,

Internal rotation paired
bones

Sense of narrowed
transverse but increased A-P
and vertical dimensions

Beginning Sensing

- When dealing with any patient it serves the physician well to remember to be mindful, respectful (of the patient's mechanism) & humble.
- Lifelong tips: Strive to be comfortable and respond immediately to your own discomfort- adjust yourself as often as needed.
- Be mindful of your patient's comfort. Adjust as needed, as often as needed.

Sensing Inherent Motion

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Exercise 1

Sensing yourself

- Patience
- Relaxed
- Courage
- Confidence
- Humility



Session Objectives

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1. Identify and locate ALL the various cranial landmarks and give examples of clinical implications of sutural restriction, cranial nerve entrapments, dural restriction and venous sinus congestion which are components of cranial somatic dysfn.
2. Compare, contrast and demonstrate different cranial contacts (holds) for dx and tx. Be able to explain proper hand/finger placement, body attitude/receptivity, comfort etc
3. Review & describe the 5 phenomena of primary respiratory mechanism (PRM) and cranial rhythmic impulse (CRI) in OCF and make your first attempts to sense them.
4. Practice palpating the PRM on others
5. Explain what happens to midline and paired parts during both phases of the PRM (inhalation/exhalation), and how that should feel during the various hand contacts (holds)
6. OA decompression treatment